

Evidence, Review Status, and Claim-Citation Addendum

Standalone rules formerly proposed as amendments to the corpus guide

Document ID	Version	Effective	Review due
NB-RAG-003	1.0	2026-08-17	2027-08-17

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Owner: Knowledge Governance Lead
Approved by: Fictional Clinical Governance Committee
Classification: Training / synthetic corpus

Document control and retrieval metadata

Field	Value
Canonical title	Evidence, Review Status, and Claim-Citation Addendum
Document ID	NB-RAG-003
Keywords	evidence status, claim type, citation, source hierarchy, review status, final answer
Authority	Knowledge Governance Lead
Applicability	Training simulations only
Relationship	Standalone extension; existing corpus documents remain unchanged

Retrieval rule

Retrieve for every review during evidence assembly, citation validation, and final status assignment.

1. Claim-type separation

Type	Definition	Authority
PATIENT_FACT	Verified structured fact from the patient record with source and observation time	Patient data source
AUTHORIZED_ORDER	Complete current signed patient-specific instruction	Authorizing clinician/order service
POLICY_REQUIREMENT	Workflow or safety constraint from current eligible policy	Approved policy
EXTERNAL_EVIDENCE	Current approved label/guideline passage with provenance	External source
MODEL_INTERPRETATION	Explanation or synthesis derived from supplied evidence	No independent authority

The UI and API must not present MODEL_INTERPRETATION as PATIENT_FACT or AUTHORIZED_ORDER.

2. Claim-level citation contract

- Every material policy, label, guideline, or patient-data claim carries a machine-readable citation ID.
- A policy citation includes document ID, version, section/heading, page/chunk identity, and effective status.
- A patient citation includes record type/ID, source, observation time, status, and retrieval time; avoid unnecessary identifiers in display.
- The validator checks that the cited passage entails the claim and is eligible/current.
- One nearby citation cannot be assumed to support an entire paragraph.
- Unsupported claims are removed or force an insufficient-evidence status.

3. Evidence sufficiency gates

Gate	Pass condition
Identity	Patient binding verified outside the LLM
Reconciliation	Medication/allergy/status discrepancies resolved or explicitly nonmaterial
Order	Complete current signed order for any final medication action
Monitoring	Required current tests/observations and named results owner
Retrieval	Applicable current sources retrieved with adequate coverage
Conflict	No unresolved hard conflict
Claims	All material claims validated against structured data/evidence
Output	Schema, safety, authorization, citation and prohibited-action checks pass

4. Status selection priority

Select the highest-severity applicable state before considering a lower-severity state. Emergency and urgent conditions bypass normal completion.

Priority	State
1	EMERGENCY_ESCALATION
2	URGENT_CLINICAL_REVIEW
3	SYSTEM_UNAVAILABLE when identity/access/required dependency blocks safe processing
4	CONFLICT_REQUIRES_REVIEW
5	INSUFFICIENT_EVIDENCE
6	DRAFT_FOR_CLINICIAN_REVIEW
7	FINAL_AUTHORIZED only when every hard gate passes

5. Final answer assembly

- Patient snapshot and timestamps.
- Attention items separated by fact, policy, evidence, and interpretation.
- Medication recommendation and explicit authorization status.
- Exact order-derived administration instructions.
- Warnings/red flags and prohibited self-adjustments.
- Tests with reason, window, booking owner, results owner, and delay/abnormal escalation.
- Appointments and future course of action with prerequisites and closure.
- Missing/stale data, conflicts, tool failures, evidence grade, citations, limitations, and safest next action.

6. Semantic similarity limitation

A high vector similarity score means a chunk resembles the query. It does not prove applicability, current status, completeness, authority, or support for a recommendation. Metadata and claim validation remain mandatory.

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